

LOMODIUM CAPSULE

MAL19913258AZ

DESCRIPTION:

CAPSULE

Capsule Colour : Green/Grey

Capsule Size : # 4

Granules Colour : White

EACH CAPSULE CONTAINS:

Loperamide Hydrochloride.....2 mg

PHARMACODYNAMICS:

Loperamide exerts a direct effect on the muscles of the intestine. It reduces the intestinal motility and thus controls the diarrhoea.

PHARMACOKINETICS:

Loperamide is not well absorbed from the gastrointestinal tract. The half-life is 9.1 hours to 14.4 hours (average 10.8 hours). Protein binding is 97%. Excretion is by renal and also appeared in faeces.

INDICATIONS:

For the control and symptomatic relief of acute non-specific diarrhoea and of chronic diarrhoea associated with inflammatory bowel disease, as well as to reduce the volume of discharge from ileostomies.

RECOMMENDED DOSE:

Usual adult dose:

Acute diarrhoea: Oral, 4 mg initially, followed by 2 mg after each unformed stool.

Chronic diarrhoea: Initial: Oral 4 mg, followed by 2 mg after each unformed stool until diarrhoea is controlled.

Maintenance: Oral, 4 to 8 mg a day in single or divided daily doses as needed.

Usual adult prescribing limits: 16 mg per day.

ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY

CONTRAINDICATIONS:

It is contraindicated in patients with known hypersensitivity to the drug and in those in whom constipation must be avoided.

WARNINGS AND PRECAUTIONS:

Not recommended for children under 6 years of age.

Its use has been associated with fatal episodes of paralytic ileus in infants and young children.

Appropriate fluid and electrolyte therapy should be given to protect against dehydration in all cases of diarrhoea. Oral rehydration therapy which is the use of appropriate fluids including oral rehydration salts remains the most effective treatment for dehydration due to diarrhoea. The intake of as much of these fluids as possible is therefore imperative.

Drug-induced inhibition of peristalsis may result in fluid retention in the intestine, which may aggravate and mask dehydration and depletion of electrolytes. If severe dehydration or electrolyte imbalance is present Loperamide should be withheld until appropriate corrective therapy has been initiated.

In acute diarrhoea, if clinical improvement is not observed in 48 hours, the administration of Loperamide should be discontinued. It is probably best avoided in the treatment of acute infective diarrhoea until more is known of its safety and efficacy. Patients with inflammatory bowel disease receiving Loperamide should be carefully observed for signs of toxic megacolon.

The use of higher than the recommended doses for control of the diarrhea may cause abnormal heart rhythms and serious cardiac events leading to death. However, in adult patients receiving the recommended dosage of Loperamide, cases of syncope and ventricular tachycardia have been reported. Some of these patients were taking other drugs or had other risk factors that may have increased their risk of cardiac adverse reactions.

Abuse and misuse of Loperamide, as an opioid substitute, have been described in individuals with opioid addiction (see Overdose).

PREGNANCY AND LACTATION:

The safety of Loperamide during pregnancy has not been established and therefore the product should be avoided during this period.

Whilst the fraction of Loperamide secreted into breast milk is extremely low, caution is advised if the drug is to be given during lactation.

SIDE EFFECTS:

Adverse reactions were generally of a minor and self-limiting nature. They were more commonly observed during the treatment of chronic diarrhoea. The following patient complaints have been reported: Abdominal pain, distention or discomfort, constipation, drowsiness or dizziness, dry mouth, nausea and vomiting, tiredness, hypersensitivity reaction including skin rash.

Post-marketing Experience

Cardiac Disorders: QT/QTc interval prolongation, Torsades de Pointes, other ventricular arrhythmias, cardiac arrest, syncope, and death (see Warning and Precautions).

Gastrointestinal disorders

Frequency 'not known': Acute pancreatitis

SYMPTOMS AND TREATMENT OF OVERDOSE:

Overdosage in man may result in constipation, CNS depression, and GI irritation.

Treatment: If vomiting occurs spontaneously upon ingestion, a slurry of 100 g of Activated Charcoal should be administered orally as soon as fluids can be retained. If vomiting has not occurred, gastric lavage should be performed followed by administration of 100 g of the Activated Charcoal slurry through the gastric tube. In the event of overdosage, patients should be monitored for signs of CNS depression for at least 24 hours. If CNS depression is observed, Naloxone may be administered. If responsive to Naloxone, vital signs must be monitored carefully for recurrence of symptoms of drug overdose for at least 24 hours after the last dose of Naloxone. In view of the prolonged action of Loperamide and the short duration (one to three hours) of Naloxone, the patients must be monitored closely and treated repeatedly with Naloxone as indicated.

In individuals who have intentionally ingested overdoses (reported in doses from 40 mg up to 792 mg per day) of Loperamide HCl, prolongation of the QT/QTc interval, Torsades de pointes, other ventricular arrhythmias and cardiac arrest, have been observed (see Warning and Precautions). Fatal cases have also been reported.

Abuse, misuse and/or overdose with excessively large doses of Loperamide, may unmask Brugada syndrome.

PACKING/PACK SIZE(S):

Blister pack of 10x10's and 100x10's.

**JAUHI UBAT DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN**

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

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P11PC L001-03
Latest Revision: Jan 2024